

Chemotherapy for anal cancer

Chemotherapy uses drugs to treat cancer and it is a common treatment for anal cancer. It is often given with radiotherapy (called chemoradiation).

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Anal cancer chemotherapy

[Chemotherapy](#) uses anti-cancer (cytotoxic) drugs to destroy cancer cells. It is sometimes called systemic anti-cancer therapy (SACT). Chemotherapy drugs disrupt the way cancer cells grow and divide. But they also affect normal cells.

For people with [anal cancer](#), chemotherapy is usually given in combination with radiotherapy. This is called [chemoradiation](#).

Chemotherapy can also be used on its own if the cancer has spread to other parts of the body. It may help control the cancer and improve symptoms.

Chemotherapy drugs for anal cancer

The most commonly used chemotherapy drugs for anal cancer are:

- [mitomycin](#)
- [fluorouracil \(5FU\)](#)
- [capecitabine \(Xeloda[®]\)](#).

Mitomycin is usually given with fluorouracil. Sometimes capecitabine is used instead of fluorouracil.

Other drugs that may be used are:

- [cisplatin](#)
- [carboplatin](#)
- [paclitaxel](#).

How chemotherapy is given

Chemotherapy drugs for anal cancer are usually given by injection into a vein (intravenously). Some people have chemotherapy as a tablet.

Chemotherapy given into a vein goes directly into your blood which carries it to all areas of your body. Chemotherapy given as a tablet is absorbed into the blood and also reaches all areas of your body.

Before you start treatment, you may have a blood test to check whether you have low levels of an enzyme called DPD. People who have low DPD levels can have more serious side effects with 2 of the chemotherapy drugs used to treat anal cancer. These are [fluorouracil \(5FU\)](#) and [capecitabine](#).

You might have chemotherapy through 1 of the following:

- a [cannula](#) – a short, thin tube the nurse puts into a vein in the arm or hand
- a [central line](#) – a fine tube that goes under the skin of the chest and into a vein close by
- a [PICC line](#) – a fine tube that is put into a vein in the arm and goes up into a vein in the chest
- an [implantable port](#) (portacath) – a disc that is put under the skin on the chest or arm and goes into a vein in the chest.

When your cannula, line or port is in place, a chemotherapy nurse will give you the drugs through it. This may be as an injection, as a drip (infusion) or through a pump.

Sometimes you have chemotherapy through a small, portable pump attached to your central line, PICC line or port. The pump controls the amount of chemotherapy going into your bloodstream over a set period of time. You can go home with the pump so you spend less time in hospital.

You usually have chemotherapy as an outpatient. Some people may need to be in hospital for a few days to have their treatment.

We have more information about [having chemotherapy](#).

Getting support

Macmillan is here to support you. If you would like to talk, you can:

- Call the Macmillan Support Line for free on [0808 808 00 00](#).
- [Chat to our specialists online](#).
- Visit our [chemotherapy forum](#) to talk with people who have had chemotherapy, share your experience, and ask questions.

Possible side effects of chemotherapy

Chemotherapy drugs can cause side effects. But chemotherapy can also help relieve symptoms of anal cancer. So you may feel better with chemotherapy, even if you have some side effects.

Your cancer doctor or nurse will tell you more about what to expect. Tell them about any side effects you have. Side effects can usually be well controlled with medicines.

We describe some of the common side effects on this page. We have more information about [chemotherapy side effects](#).

Although they can be hard to cope with, side effects usually get better after your treatment finishes.

Risk of infection

Chemotherapy can reduce the number of white blood cells in your blood. These cells fight infection. If your white blood cell count is low, you may be more likely to get an [infection](#). A low white blood cell count is called neutropenia.

An infection can be very serious when the number of white blood cells is low. It is important to get any infection treated as soon as possible. If you have any of the following symptoms, contact the hospital straight away on the 24-hour number you have been given:

- a temperature above 37.5°C
- a temperature below 36°C
- you feel unwell, even with a normal temperature
- you have symptoms of an infection.

Symptoms of an infection include:

- feeling shivery and shaking
- a sore throat
- a cough
- breathlessness
- diarrhoea
- needing to pass urine (pee) a lot, or discomfort when you pass urine..

It is important to follow any specific advice your cancer treatment team gives you.

Your white blood cell count will usually return to normal before your next treatment. You will have a blood test before having more treatment. If your white blood cell count is low, your doctor may delay your treatment for a short time, until your cell count increases.

Bruising and bleeding

Chemotherapy can reduce the number of platelets in your blood. Platelets are cells that help the blood to clot.

If the number of platelets is low, you may bruise or bleed easily. You may have:

- nosebleeds
- bleeding gums
- heavy periods
- blood in your urine (pee) or stools (poo)
- tiny red, brown or purple spots on the skin that may look like a rash.

If you have any unexplained bruising or bleeding, contact the hospital straight away on the 24-hour number. You may need a drip to give you extra platelets. This is called a platelet transfusion.

Anaemia (low number of red blood cells)

Chemotherapy can reduce the number of red blood cells in your blood. Red blood cells carry oxygen around the body. If the number of red blood cells is low, this is called anaemia. You may feel:

- very low in energy
- breathless
- dizzy and lightheaded.

If you have these symptoms, contact the hospital straight away on the 24-hour number. You may need treatment for anaemia. If you are very anaemic, you may need a drip to give you extra red blood cells. This is called a blood transfusion.

Diarrhoea

Some chemotherapy drugs can cause [diarrhoea](#). Diarrhoea means passing more stools than is usual for you, or having watery or loose stools. If you have a [stoma](#), it will be more active than usual.

Sometimes diarrhoea can be severe. Your hospital team may give you anti-diarrhoea drugs to take at home. It is important to follow any advice from them about taking anti-diarrhoea drugs. Contact the hospital on the 24-hour number straight away if:

- you have diarrhoea at night
- you have uncomfortable stomach cramps
- you have diarrhoea 4 or more times in a day
- you have a moderate increase in stoma activity
- the anti-diarrhoea drugs do not work within 24 hours.

If you are taking [capecitabine](#) tablets, the hospital may ask you to stop taking them. They will tell you when you can start taking them again.

Feeling sick

Your doctor, nurse or pharmacist will prescribe anti-sickness drugs to help prevent or control [sickness](#). Take the anti-sickness drugs exactly as they tell you, even if you do not feel sick. It is easier to prevent sickness than to treat it after it has started.

Sore mouth

This treatment may cause a [sore mouth and throat](#). You may also get mouth ulcers. This can make you more likely to get a mouth or throat infection. Use a soft toothbrush to clean your teeth or dentures in the morning, at night and after meals. Contact the hospital straight away on the 24-hour number if:

- your mouth or throat is sore, or affecting how much fluid you can drink or food you can eat
- your mouth, tongue, throat or lips have any blisters, ulcers or white patches.

Feeling tired

[Feeling tired](#) is a common side effect. It is often worse towards the end of treatment and for some weeks after it ends. Try to pace yourself and plan your day so you have time to rest. Gentle exercise, like short walks, can give you more energy. If you feel sleepy, do not drive or use machinery.

Numb or tingling hands or feet

Some chemotherapy drugs can affect the nerves in your fingers and toes. This can cause [numbness, tingling or pain in your hands or feet](#). You might find it hard to do fiddly tasks such as fastening buttons or tying shoe laces. If you have these symptoms, always tell your doctor, nurse or pharmacist. They sometimes need to change the drug or the dose of the drug.

Hand-foot (palmar-plantar) syndrome

This treatment can affect the palms of your hands and the soles of your feet. This is called palmar-plantar or hand-foot syndrome.

If you have white skin these areas may become red. If you have black or brown skin, these areas might get darker.

The skin on the palms of your hands and the soles of your feet may:

- be sore
- be painful, tingle, or swell
- peel, crack or blister.

If you have any of these symptoms, contact the hospital straight away on the 24-hour number. They can give you advice. This is especially important if you have any broken skin or if walking is difficult. They can prescribe creams and painkillers to help.

Effects on the heart

[Fluorouracil](#) and [capecitabine](#) can affect [how the heart works](#). You may have tests to see how well your heart is working. These may be done before, during and after treatment.

Contact the hospital straight away on the 24-hour number if you have any of these symptoms during or after treatment:

- breathlessness
- dizziness
- changes to your heartbeat
- swollen feet and ankles.

Always call 999 if you have:

- chest pain, pressure, heaviness, tightness or squeezing across the chest
- difficulty breathing.

Effects on sex life

Having chemotherapy can sometimes affect your sex life. Side effects like tiredness and feeling sick can reduce your sex drive and make having sex difficult.

If you have a partner, try to tell them how you feel. It can also help to talk with your doctor or specialist nurse about your concerns. They can reassure you and support you with any problems. We have more information about [cancer and your sex life](#).

Contraception

Your doctor, nurse or pharmacist will advise you not to get pregnant or make someone pregnant while having this treatment and for some time afterwards. The drugs may harm a developing baby. It is important to use contraception to prevent pregnancy. Follow their advice about:

- what types of contraception to use
- how long after treatment you should continue to use contraception.

Fertility

Some cancer drugs can affect whether you can get pregnant or make someone pregnant. If you are worried about this, it is important to talk with your doctor before you start treatment.

We have more information about [fertility and cancer](#).

About our information

This information has been written, revised and edited by Macmillan Cancer Support's Cancer Information Development team. It has been reviewed by expert medical and health professionals and people living with cancer.

References



Below is a sample of the sources used in our anal cancer information. If you would like more information about the sources we use, please contact us at informationproductionteam@macmillan.org.uk

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Our cancer information meets the [PIF TICK](#) quality mark.

This means it is easy to use, up-to-date and based on the latest evidence. Learn more about [how we produce our information](#).

The language we use

We want everyone affected by cancer to feel our information is written for them.

We want our information to be as clear as possible. To do this, we try to:

- use plain English
- explain medical words
- use short sentences
- use illustrations to explain text
- structure the information clearly
- make sure important points are clear.

We use gender-inclusive language and talk to our readers as 'you' so that everyone feels included. Where clinically necessary we use the terms 'men' and 'women' or 'male' and 'female'. For example, we do so when talking about parts of the body or mentioning statistics or research about who is affected.

You can read more about how we produce our information [here](#).

How we can help

Clinical Information Nurse Specialists

Our Cancer Information Nurse Specialists are dedicated cancer nurses available to talk to on our Macmillan Cancer Support Line.



Call us free on 0808 808 0000

8am to 8pm, 7 days a week. You can talk to our cancer specialists about anything.



Email the Macmillan Support Line

Email the Macmillan Support Team if you have any questions.

Chat online

Available 8am to 7.45pm, 7 days a week. For confidential, real-time support if you don't want to call.



Anal cancer forum

Join our anal cancer forum and chat anonymously to others who understand what you are going through. Our community is available 24/7. It is a safe space where you can share experiences, ask questions and talk to people who understand.



Find cancer support services in your area

What's going on near you? Find out about support groups, where to get information and how to get involved with Macmillan where you live.

Get help